

Surgical Pathology Report

Name: [REDACTED]
DOB: [REDACTED]
Gender: F
MRN: [REDACTED]
Location:
Physician:

Case #: [REDACTED]
Collected:
Received:
Reported:
Copy To:

Pathologic Interpretation:

- A. **SENTINEL NODE #1:**
- No carcinoma seen in one lymph node (0/1).
- Immunohistochemistry for keratin is pending.

- B. **SENTINEL NODE #2:**
- No carcinoma seen in one lymph node (0/1).
- Immunohistochemistry for keratin is pending.

- C. **SENTINEL NODE #3:**
- No carcinoma seen in two lymph nodes (0/2).
- Immunohistochemistry for keratin is pending.

D. **RIGHT BREAST, ONE STITCH SUPERIOR, TWO STITCHES LATERAL:**

- INFILTRATING DUCTAL CARCINOMA, moderately differentiated, Nottingham grade 2 (2+3+2=7), 2.0 cm in greatest dimension.
- Margins are free of invasive carcinoma
- Lymphovascular invasion is not identified.
- Prior biopsy site
- See tumor summary.

- DUCTAL CARCINOMA IN SITU, high nuclear grade (DIN 3) with expansive comedo necrosis, solid type, present in 2/10 slides and representing less than 5% of submitted tissue.
- Margins free of DCIS.

- E. **NON-SENTINEL LYMPH NODE:**
- No carcinoma seen in two lymph nodes (0/2).

- F. **SENTINEL NODE #4:**
- No carcinoma seen in one lymph node (0/1).
- Immunohistochemistry for keratin is pending.

- G. **SENTINEL NODE #5:**
- No carcinoma seen in one lymph node (0/1).
- Immunohistochemistry for keratin is pending.

- H. **SENTINEL NODE #6:**
- No carcinoma seen in one lymph node (0/1).
- Immunohistochemistry for keratin is pending.

- I. **SENTINEL NODE #7:**
- No carcinoma seen in one lymph node (0/1).
- Immunohistochemistry for keratin is pending.

- J. **SENTINEL NODE #8:**
- No carcinoma seen in one lymph node (0/1).
- Immunohistochemistry for keratin is pending.

TUMOR SUMMARY

Specimen:

- Total breast (including nipple and skin)

Procedure:

- Total mastectomy (including nipple and skin)

Lymph Node Sampling:

ICD-O-3

Carcinoma, infiltrating ductal, NOS 8500/3
Site: breast, NOS C50.9 3/12/11 lw

UUID:0D24479F-2F86-43AC-AAB1-83AAD99542C4

TCGA-EW-A1PF-01A-PR

Redacted



- Sentinel lymph node(s)
- Axillary dissection (partial or complete dissection)

Specimen Integrity:

- Single intact specimen (margins can be evaluated)

Specimen Laterality:

- Right

Tumor Site: Invasive Carcinoma

- Upper outer quadrant
- Central

Tumor Size: Size of Largest Invasive Carcinoma

- Greatest dimension of largest focus of invasion over 0.1 cm: 2.0 cm
- Additional dimensions: 2.0 x 1.0 cm

Tumor Focality:

- Single focus of invasive carcinoma

Macroscopic and Microscopic Extent of tumor:

- Skin: Invasive carcinoma does not invade into the dermis or epidermis
- Skeletal muscle: Skeletal muscle is present and is free of carcinoma

Ductal Carcinoma In Situ (DCIS):

- DCIS IS PRESENT
- Extensive intraductal component (EIC) negative

Size (Extent) of DCIS:

- Estimated size (extent) of DCIS (greatest dimension using gross and microscopic evaluation) is at least: 5 cm

*Number of blocks with DCIS: 2

*Number of blocks examined: 10

- Architectural Patterns: Solid
- Nuclear Grade: Grade III (high)
- Necrosis: Present, central (expansive "comedo" necrosis)

Lobular Carcinoma In Situ (LCIS)

- Not identified

Histologic Type of Invasive Carcinoma:

- Invasive ductal carcinoma (no special type or not otherwise specified)

Histologic Grade: Nottingham Histologic Score

Glandular (Acinar)/Tubular Differentiation: Score 2

Nuclear Pleomorphism: Score 3

Mitotic Count: Score 2

Overall Grade: Grade 2: score 7

Margins:

- Margins uninvolved by invasive carcinoma
Distance from closest margin: 6 mm (deep)
- Margins uninvolved by DCIS
Distance from closest margin: 14 mm (deep)

Treatment Effect: Response to Presurgical (Neoadjuvant) therapy:

- In the Breast: No known presurgical therapy
- In the Lymph Nodes: No known presurgical therapy

Lymph-Vascular Invasion: Not identified.

*Dermal Lymph-Vascular Invasion: Not identified

Lymph Nodes:

Number of sentinel lymph nodes examined: 9

Total number of lymph nodes examined (sentinel and nonsentinel): 11

Number of lymph nodes with macrometastases (>0.2 cm): 0

Number of lymph nodes with micrometastases (>0.2 mm to 0.2 cm and/or >200 cells): 0

Number of lymph nodes with isolated tumor cells (≤0.2 mm and ≤200 cells): 0

*Method of Evaluation of Sentinel Lymph Nodes: Hematoxylin and eosin (H&E), one level
Immunohistochemistry pending

Pathologic Staging (pTNM): pT1c N0 Mx

Primary Tumor: pT1c

Regional Lymph Nodes: pN0

Distant Metastasis: Not applicable

Ancillary Studies:

Estrogen Receptor:

- Performed on another specimen ; Results: Immunoreactive tumor cells present (≥1%)

Progesterone Receptor:

- Performed on another specimen ; Results: Immunoreactive tumor cells present (≥1%)

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Her2:

- Performed on another specimen ; Results: Negative

AJCC classification (7th ed) (pTNM): pT1c N0 Mx

NOTE: Some immunohistochemical antibodies are analyte specific reagents (ASRs) validated by our laboratory. These ASRs are clinically useful indicators that do not require FDA approval. These clones are used: ID5-ER, PgR 636-PR, A485-HER2, H-11-EGFR. All immunohistochemical stains are used with formalin or molecular fixed, paraffin embedded tissue. Detection is by Envision Method. The results are read by a pathologist as positive or negative.

As the attending pathologist, I attest that I: (i) Examined the relevant preparation(s) for the specimen(s); and (ii) Rendered the diagnosis(es).

*****Electronically Signed Out By*****

**Procedures/Addenda
Addendum**

**Date Ordered:
Date Complete:
Date Reported:**

Status: Signed Out

Addendum Diagnosis

A: Immunohistochemistry for keratin is negative.
B: Immunohistochemistry for keratin is negative.
C: Immunohistochemistry for keratin is negative.
F: Pending Immunohistochemistry (keratin).
G: Immunohistochemistry for keratin is negative.
H: Immunohistochemistry for keratin is negative.
I: Immunohistochemistry for keratin is negative.
J: Immunohistochemistry for keratin is negative.

Intraoperative Consultation

A. Sentinel node #1, FS: No carcinoma seen
B. Sentinel node #2, FS: No carcinoma seen
C. Sentinel node #3, FS: No carcinoma seen
F. Sentinel node # 4, FS: No carcinoma seen
G. Sentinel node # 5, FS: No carcinoma seen
H. Sentinel node #6, FS: No carcinoma seen
I. Sentinel node #7, FS: No carcinoma seen
J. Sentinel node #8, FS: No carcinoma seen

MD

Clinical History:

None provided

Operation Performed

Right total mastectomy with sentinel node and possible axillary dissection

Pre Operative Diagnosis:

Right breast carcinoma

Specimen(s) Received:

- A: Sentinel node #1, FS
- B: Sentinel node #2, FS
- C: Sentinel node #3, FS
- D: Right breast, one stitch superior, two stitches lateral
- E: Non sentinel lymph node
- F: Sentinel node # 4, FS
- G: Sentinel node # 5, FS
- H: Sentinel node #6, FS
- I: Sentinel node #7, FS
- J: Sentinel node #8, FS

Gross Description:

- A. Received fresh is a yellow-tan tissue, measuring 1.6 x 1.1 x 0.2 cm. Bisected and submitted in toto in one cassette for frozen section.
- B. Received fresh is a tan-yellow tissue, 1.9 x 0.9 x 0.3 cm. Bisected and submitted in toto in one cassette for frozen section.
- C. Received fresh are fragments of tan tissue, 3.1 x 2.1 x 0.3 cm. Sections are submitted as follows:
 - 1 Possible lymph node, bisected, for frozen section
 - 2 Remaining tissue in toto
- D. Received in formalin is a total mastectomy weighing 450 grams, measuring 20.0 x 16.0 x 2.6 cm with a portion of skin 16.5 x 7.0 cm. The areola is 2.0 cm. In the upper outer portion of the breast 0.6 cm from the deep margin and 1.6 cm posterior to the skin at approximately 11 o'clock is an ill-defined fibrotic area 2.0 x 2.0 x 1.0 cm in dimension. No other lesions are grossly identified. Resection margins inked in black. Stroma to fat ratio is 5 to 95%. Sections submitted as follows:
 - 1 Nipple (base of nipple) and tip in perpendicular sections
 - 2&3 Composite section of the ill-defined mass in relation to the deep margin and to the anterior skin (Cassette 2 contains skin, cassette 3 contains deep margin)
 - 4&5 Additional sections of lesion in relation to deep margin
 - 6&7 Additional sections of lesion in relation to surrounding breast tissue
 - 8 Section from lower outer quadrant
 - 9 Section from upper inner quadrant
 - 10 Lower inner quadrant
- E. Received in formalin are multiple fragments of adipose tissue, 1.6 x 1.3 x 0.4 cm. The specimen is submitted in toto in one cassette.
- F. Received fresh is a tan-red tissue fragment, 0.9 x 0.6 x 0.2 cm. The specimen is bisected and submitted in toto in one cassette for frozen section.
- G. Received fresh is a tan tissue fragment, 1.1 x 0.6 x 0.4 cm. The specimen is bisected and submitted in toto in one cassette for frozen section.
- H. Received fresh is tan tissue fragment, 1.1 x 0.6 x 0.3 cm. The specimen is bisected and submitted in toto in one cassette for frozen section.
- I. Received fresh is a tan tissue fragment, 0.6 x 0.5 x 0.3 cm. The specimen is bisected and submitted in toto in one cassette for frozen section.

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J.

Received fresh is a tan tissue fragment, 0.7 x 0.4 x 0.3 cm. The specimen is submitted in toto in one cassette for frozen section.

Criteria	Yes	No
Diagnosis Discrepancy		/
Primary Tumor Site Discrepancy		/
IIA Discrepancy		/
Prior Malignancy History		/
Qual/Synchronous Primary Noted		/
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	Date Reviewed: 3/12/11	